Advanced MET exon 14 skipping lung cancer National Cancer Control Programme (NCCP) 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

BASELINE → TREATMENT → FOLLOW-UP

Checks alongside continuous tablets

Relative stages, not equal time periods or booked appointments. NCCP does not specify a universal number of weeks between these checks; your team agrees the schedule.

Check	Before the first dose	Starting daily treatment	Ongoing reviews	Benefit or toxicity review	Change or stopping
Cancer and eligibility	MET test and previous treatment	Confirm prescription	Scans under the Consultant plan	Review response and symptoms	Agree the next plan
Blood and organ function	Blood count, electrolytes, kidney and liver tests	Review results and early symptoms	Repeat regular tests	Extra tests for abnormalities	Follow unresolved effects
Heart	electrocardiogram (ECG) when indicated	Check risk and interacting medicines	ECG/electrolytes when indicated	Assess fainting or palpitations urgently	Confirm the follow-up plan
Symptoms and practical support	Discuss lungs, swelling and bowel plan	Take with food; check supply	Review symptoms, weight and adherence	Authorised hold or dose decision	Continue contact advice and contraception

THE WHOLE TREATMENT JOURNEY

Tepotinib monotherapy: treatment and checks

Advanced MET exon 14 skipping non-small-cell lung cancer (NSCLC) · after prior systemic therapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Starting daily treatment	Ongoing reviews	Benefit or toxicity review	Change or stopping
Treatment NCCP 00823 · SMPC	Confirm prescription	Daily tepotinib with food	Continue authorised daily dose	Discuss benefit and side effects	Team authorises change or stopping
Cancer and eligibility NCCP 00823 · SMPC	MET test and previous treatment	Confirm prescription	Scans under the Consultant plan	Review response and symptoms	Agree the next plan
Blood and organ function NCCP 00823 · SMPC	Blood count, electrolytes, kidney and liver tests	Review results and early symptoms	Repeat regular tests	Extra tests for abnormalities	Follow unresolved effects
Heart NCCP 00823 · SMPC	ECG when indicated	Check risk and interacting medicines	ECG/electrolytes when indicated	Assess fainting or palpitations urgently	Confirm the follow-up plan
Symptoms and practical support NCCP 00823 · SMPC	Discuss lungs, swelling and bowel plan	Take with food; check supply	Review symptoms, weight and adherence	Authorised hold or dose decision	Continue contact advice and contraception

What each check is for

1 Confirm eligibility

Before the first dose

Treatment depends on a specific tumour alteration and previous treatment.

Ask / act: Document the NCCP 00823a indication and agreed prescription.

Sources: NCCP 00823

2 Keep tests and symptoms connected

At scheduled reviews and between visits

Blood tests can change before symptoms and symptoms can occur between appointments.

Ask / act: Confirm who orders tests and reviews results; give an urgent contact plan.

Sources: NCCP 00823 · SMPC

3 Possible interstitial lung disease (ILD) or liver injury

At any time

Suspected lung inflammation and significant liver abnormalities can require an immediate hold.

Ask / act: Urgent clinical assessment and an authorised treatment decision.

Sources: NCCP 00823 · SMPC

4 Benefit or toxicity changes

At treatment reviews

There is no fixed chemotherapy course length.

Ask / act: Agree continuation, an authorised hold/restart, or the next treatment plan.

Sources: NCCP 00823 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.25 · Sources checked 2026-10-29

Source reconciled; independent clinical, pharmacy and patient/PPI review pending